Clinical Study Summary Report (CSSR) Document Status: Draft | **Version:** 1.0 | **Date:** 07-Apr-2026

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1. Administrative & Study Identification

Full Study Title: A Phase III Randomized, Open-Label Active Comparator-Controlled Multicenter Study to Evaluate Efficacy and Safety of Obinutuzumab in Patients With Primary Membranous Nephropathy **Brief Study Title:** A Study Evaluating the Efficacy and Safety of Obinutuzumab in Participants With Primary Membranous Nephropathy **Short Title/Acronym:** MAJESTY **Protocol Number:** WA41937 **NCT Number:** NCT04629248 **Posting ID:** 6011073200583673332 **Sponsor/Company Name:** Hoffmann-La Roche (Genentech) / Roche **Investigational Product:** Obinutuzumab (Gazyva / Gazyvaro) **Phase of Development:** Phase III **Trial Status:** ACTIVE_NOT_RECRUITING (Active but not currently recruiting) **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead

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2. Study Synopsis & Rationale

2.1 Study Objective * **Primary Objective:** To evaluate the efficacy of obinutuzumab compared with tacrolimus in achieving complete remission in participants with primary membranous nephropathy (pMN) at week 104. * **Secondary Objectives:** To evaluate the percentage of participants who achieve an overall remission (complete or partial) at week 104, complete remission at week 76, time to treatment failure, meeting escape criteria, or relapse, and to assess the safety, pharmacodynamics, and pharmacokinetics (PK) of obinutuzumab.

2.2 Background & Rationale Primary membranous nephropathy is a chronic autoimmune condition that causes immune-mediated injury to the glomeruli, leading to proteinuria and a progressive decline in renal function. Up to 30% of people with pMN will develop kidney failure over 10 years despite current treatment approaches. This study is being conducted to evaluate if obinutuzumab—a type II anti-CD20 monoclonal antibody designed to achieve deep tissue B-cell depletion—can help more patients achieve complete remission, maintain kidney function, and delay the onset of life-threatening complications compared to tacrolimus, in a disease area that historically has had limited approved treatment options.

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3. Study Design & Methodology

3.1 Design Framework * **Study Type:** Interventional * **Control Type:** Active Comparator-Controlled (Tacrolimus) * **Blinding:** Open-label * **Randomization:** 1:1 ratio

3.2 Planned Enrollment & Duration * **Target N:** 142 adult participants * **Number of Sites:** 53 global locations * **Estimated Study Start:** 25-Jun-2021 * **Estimated Primary Completion:** 09-Jan-2025

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 to ≤ 75 years old at the time of signing the Informed Consent Form. 2. Diagnosis of primary membranous nephropathy (pMN) according to renal biopsy prior to or during screening. 3. Screening urinary protein-to-creatinine ratio (UPCR) ≥ 5 g/g from 24-hour urine collection after best supportive care for ≥ 3 months prior to screening OR screening UPCR ≥ 4 g/g after best supportive care for ≥ 6 months prior to screening. 4. Estimated Glomerular Filtration Rate (eGFR) ≥ 40 mL/min/1.73m^2 or qualified endogenous creatinine clearance ≥ 40 mL/min/1.73m^2 based on 24-hour urine collection during screening. 5. Other inclusion criteria may apply.

4.2 Exclusion Criteria 1. Participants with a secondary cause of membranous nephropathy. 2. Evidence of $\ge 50\%$ reduction in proteinuria during the previous 6 months prior to randomization. 3. Severe renal impairment, including the need for dialysis or renal replacement therapy. 4. Receipt of an excluded therapy, including any anti-CD20 therapy less than 9 months prior to or during screening; or cyclophosphamide, tacrolimus, or cyclosporin less than 6 months prior to or during screening. 5. Type 1 or 2 diabetes mellitus (steroid-induced diabetes without evidence of diabetic nephropathy on renal biopsy is permitted). 6. Pregnancy or breastfeeding. 7. Significant or uncontrolled medical disease which, in the investigator's opinion, would preclude participant participation. 8. Known active infection of any kind or recent major episode of infection. 9. Major surgery requiring hospitalization within the 4 weeks prior to screening. 10. Current active alcohol or drug abuse or history of alcohol or drug abuse within 12 months prior to screening. 11. Intolerance or contraindication to study therapies. 12. Other exclusion criteria may apply.

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5. Intervention & Treatment Plan

5.1 Investigational Regimen * **Dosage/Frequency:** * *Experimental Arm:* Obinutuzumab administered according to region and anti-PLA2R autoantibody titer. * *Active Comparator Arm:* Tacrolimus at a starting oral dose of 0.05 mg/kg/day divided into two equal doses (12-hour intervals), titrated to a serum trough level of 5-7 ng/mL. Optimized dose is maintained for a maximum of 52 weeks depending on response, then tapered over 8 weeks. * **Route of Administration:** Obinutuzumab (IV Infusion) / Tacrolimus (Oral) *

Duration of Treatment: Up to 104 weeks (2 years) for primary endpoint evaluation.

5.2 Concomitant & Prohibited Therapy * **Permitted:** Best supportive care, including renin-angiotensin system blockade with an ACE inhibitor. * **Prohibited:** Concurrent immunosuppressants outside the study protocol, other B-cell depleting agents, or live virus vaccinations during the active treatment phase.

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6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Up to Day -1	Informed Consent, Renal Biopsy Review, 24-hour Urine Collection (UPCR), eGFR, Anti-PLA2R Titer
Baseline	Day 0	Randomization (1:1), First Dose of Obinutuzumab or Tacrolimus
Interim	Week 52 / Week 76	Evaluation of complete remission rates, Tacrolimus tapering (if applicable), Safety monitoring
End of Study (Primary)	Week 104	Final assessment of complete and overall remission, Safety Labs

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7. Outcome Measures (Endpoints)

7.1 Primary Endpoint * **Definition:** Percentage of participants who achieve a Complete Remission (CR) at Week 104 (2 years). * **Measurement Method:** 24-hour urine collection for UPCR and assessment of renal function (eGFR).

7.2 Secondary & Exploratory Endpoints * Percentage of participants who achieve an Overall Remission (Complete or Partial Remission) at Week 104. * Percentage of participants who achieve a Complete Remission at Week 76. * Time to Treatment Failure, Meeting Escape Criteria, or Relapse after Complete or Partial Remission.

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8. Safety & Adverse Event Reporting

- **Adverse Event (AE) Monitoring:** Standard collection of AEs, with particular monitoring for known obinutuzumab-associated events (infusion-related reactions, leukopenia, weakened immune system) and tacrolimus-associated events (high blood sugar, hypertension, kidney issues, elevated potassium).
- **Serious Adverse Events (SAEs):** Must be reported within 24 hours of site awareness. Safety findings throughout the trial were consistent with the well-characterized profile of obinutuzumab, and no new safety signals were identified.
- **Data Monitoring Committee (DMC):** Internal and external safety monitoring maintained by Hoffmann-La Roche.

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9. Statistical Analysis Plan (SAP) Summary

- **Analysis Populations:** Intent-to-Treat (ITT) population for primary efficacy outcomes ($N=142$). Safety Set for all participants receiving at least one dose of study medication.
- **Sample Size Justification:** The trial was powered to detect a statistically significant and clinically meaningful superiority of obinutuzumab over tacrolimus in achieving complete remission at week 104.
- **Handling of Missing Data:** Participants who discontinue treatment prematurely or do not have evaluable week 104 UPCR data are typically classified as non-responders for the primary efficacy endpoint.

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10. Quality Assurance & Regulatory Compliance

- **GCP Compliance:** This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.
- **Monitoring Plan:** Routine clinical site monitoring, centralized data review, and regular verification of endpoints including independent laboratory analysis of UPCR and anti-PLA2R titers.
- **Audit Trail:** Data entered into eCRFs with thorough clarification tracking to maintain dataset integrity.

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11. Study Identifiers & Governance

Primary ID: {{WA41937}} **Posting ID:** {{601107320058367332}} **Registry Number:** {{NCT04629248}} **Sponsor/Lead Organization:** {{Hoffmann-La Roche / Roche}} **Brief Study Title:** {{A Study Evaluating the Efficacy and Safety of Obinutuzumab in Participants With Primary Membranous Nephropathy}} **Official Regulatory Title:** {{A Phase III Randomized, Open-Label Active Comparator-Controlled Multicenter Study to Evaluate Efficacy and Safety of Obinutuzumab in Patients With Primary Membranous Nephropathy}} **Trial Status:** {{ACTIVE_NOT_RECRUITING - Active but not currently recruiting}}

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12. Clinical Framework (Primary Membranous Nephropathy Specific)

Primary Condition / Disease Area: {{Primary Membranous Nephropathy (pMN)}} **Diagnosis Threshold:** {{UPCR >= 5 g/g from 24-hour urine collection after best supportive care for >= 3 months or UPCR >= 4 g/g after >= 6 months}} **Medical Methodology:** {{B-cell-targeted therapy (Anti-CD20) vs CNI}} **Optimization Target:** {{Complete remission at week 104}}

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13. Study Procedures & Methodology

Management Pathway: {{Targeted B-cell depletion or CNI based immunosuppression}} **Education Protocol (HCP):** {{Standard Investigator Training & Site Initiation Visits}} **Education Protocol (Patient):** {{Informed Consent Discussion & Dosing Education}} **Quality Audit Mechanism:** {{Central laboratory analysis for UPCR and anti-PLA2R antibodies}}

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14. Observation & Follow-Up Schedule

Total Duration: {{104 weeks for primary endpoint, up to 2028 for study completion}} **Onsite Visit Cadence:** {{Weeks 52, 76, 104}} **Remote Monitoring Frequency:** {{Standard interval monitoring per site protocol}} **Checklist Review Interval:** {{Standard interval monitoring per site protocol}}

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15. Sign-off & Approval

| Role | Name | Signature | Date | | :--- | :--- | :--- | :--- | | **Principal Investigator** | [Name] | ____ **| [DD-MMM-YYYY] | | Clinical Operations Lead | [Name] |** ___ **|** ***[DD-MMM-YYYY] | | Lead Statistician | [Name] |*** _____ | [DD-MMM-YYYY] |

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16. Appendix: System Metadata & Additional Mappings

- **Posting ID:** 601107320058367332
- **Preferred UMLS Name:** Primary Membranous Nephropathy
- **Analyzed Drug/Substance Mappings:** caffeine
- **ATC Codes:** N06BC01, D11AX26
- **Trade Names:** Belcomp, Migracet, Bayer Headache Relief, Bayer Migraine, Arthriten Inflammatory Pain, Sedalmex, Orbivan, Capacet, Arthriten, BC Arthritis, BC Original Formula, Pamprin Max Formula, Exaprin, Stanback Headache Powder Reformulated Jan 2011, Painaid ESF, Farbital, Backprin, Prodrin, BC Headache, Flextra Plus, Stanback Headache Powder, Ascomp, Bayer Back and Body Pain, Excedrin Quick Tab, Lucidex, Zerlor, Cafgesic, Anacin Advanced Headache Formula, Trezix, Pep-Back, Dolgic Plus, MigraTen, Panlor DC Reformulated Jan 2008, Durabac Forte, Excedrin Tension Headache, Valorin Extra, Levacet, Quala Cet, Durabac, Dolgic LQ, Migralam, Migrend, Cafcit, Orphengesic, Nonbac, Phrenilin with Caffeine and Codeine, Panlor SS, Ex-Pain, Panadol Extra, No Doz, Triad, Combiflex, Fioricet, Alagesic, Americet, Anacin, Anolor, Anoquan, Arcet, Aspircaf, Bayer Select, CP-2, Dolmar, Emagrin, Endolor, Esgic, Excedrin, Ezol, Fiorinal with Codeine, Fioricet with Codeine, Femcet, Fiortal, Fiortal with Codeine, Fiormor, Fiorpap, Fortabs, G-1, Genace brand of acetaminophen/aspirin/caffeine, Genasan, Geone, Hycomine

Compound, Ide-cet, Idenal, Isocet, Isollyl, Keep Alert, Laniroif, Major-Cin, Margesic, Medigesic, Migergot, Minotal, Mygracet, Norgesic, Pacaps, P-A-C Analgesic, PC-CAP, Pharmagesic, Phrenilin Forte, Propoxyphene Compound 65, Q-Acin, Caffedrine, Repan, Rid-A-Pain, Saleto, Scot Tussin Original, Stat Awake, Stay Alert, Supac, Synalgos-DC, Tenake, Tencet, Tussirex, Two-Dyne, Uni-Case, Uni-Ann, Vanquish, Verv, Vivarin, Waykup, Wakespan, Wigraine, Zebutal, Revive brand of caffeine, Cafergot, Fiorinal, NoSnooze, Vtol, Dvorah, Arthriten Max, Sed-Max, Panlor Reformulated Jan 2018, Excedrin Mild Headache, Jet-Alert, Vanatol, Backaid IPF, Drowz-Away, Uricalm Intensive, Darvon Compound.